Batterjee Medical College
For Science and Technology

Psychiatry · Clinical Lecture

Personality Disorders

What personality is, when it becomes a disorder, the ten DSM-5-TR types across three clusters, and how to assess and manage them.

Name: Dr. Hussain M. Khrad, MBBS, ABNP, ABAM, MA
Designation: Consultant Psychiatrist, KFSHRC-Jeddah · Psychiatry Residency Program Director (SCFHS)
Department: Department of Psychiatry

Learning Objectives (1 of 2)

Mapped to the BMC Knowledge / Skill / Value framework. You should already know psychiatric history taking and the mental status examination.

Knowledge & understanding

- Distinguish personality, personality TRAIT and personality DISORDER
- State the DSM-5-TR general criteria for a personality disorder
- Name the three clusters and the ten disorders within them

Skills

- Take a history that separates a trait from a disorder
- Tell a stable pattern apart from a state: mood, substance, psychosis
- Recognise borderline personality disorder and assess its risk

Values & attitudes

- See the disorder as suffering, not as a bad character
- Judge a pattern against the patient's OWN culture, not yours
- Notice your own reaction to the patient and use it, not act on it

Learning Objectives (2 of 2)

Knowledge & understanding

- Describe the aetiology: temperament, genes and early adversity
- Outline the course and prognosis of the major disorders
- State which treatments have evidence and which do not

Skills

- Build a differential for a patient who seems “difficult”
- Decide when NOT to diagnose: in crisis, under 18, mid-intoxication
- Make a safe plan for the patient who self-harms repeatedly

Values & attitudes

- Set limits that are consistent, kind and clearly explained
- Involve the family as our setting expects, with the patient's consent
- Hold realistic hope: most of these patients improve with time

Why this matters to you: whatever specialty you enter, the patient who most tests your professionalism will often have one of these disorders, and your reaction will shape their care.

BMC
Batterjee Medical College

PART 1

What a personality disorder is

Personality · trait versus disorder · the DSM-5-TR general criteria · when it is NOT a disorder · the three clusters · how common they are.

Where this starts

A 24-year-old woman, brought to the emergency department by her sister

Noura has come to the emergency department for the fourth time this year after cutting her forearm. Each visit has followed a quarrel — this time her closest friend did not reply to a message, and Noura became convinced she was being abandoned.

Her sister says Noura's relationships are intense and short: a new friend is "perfect" one week and "a traitor" the next. She has changed her course of study three times, describes herself as feeling "empty," and her mood can swing within a single afternoon.

She is not intoxicated and is fully oriented. The cutting was not intended to end her life — "it makes the feeling stop." The nurse who saw her last month wrote "attention-seeking, no psychiatric illness" in the notes.

Hold Noura in mind for the next two hours. By the end you should be able to name her disorder, separate it from the conditions it imitates, assess her risk, and say what actually helps — and what the note in her chart got wrong.

> **Notice what is here: a pervasive pattern since adolescence — unstable relationships, identity and mood, impulsivity, emptiness, frantic responses to abandonment, and self-harm to regulate emotion. This is an illness, not a character flaw.**

The patient behind the label

Patient profile

Noura · 24 · female

Fourth emergency visit this year — cut her forearm after a quarrel.

Mood	Relationships	Identity
Swings within a single afternoon	Intense, short — "perfect", then "traitor"	Unstable; chronic emptiness

"The self-harm makes the feeling stop."

Not a suicide attempt — it regulates unbearable emotion. A pervasive pattern since adolescence.

An illness — not attention-seeking. Assess the risk, don't dismiss it.

THE NOTE IN HER CHART

✗ WRONG

~~*"Attention-seeking. No psychiatric illness."*~~

Why that note is dangerous

- This is an illness, not a character flaw — an enduring, pervasive, impairing pattern, present since adolescence.
- Borderline personality disorder carries an 8–10% lifetime suicide risk; chronic self-harm RAISES that risk, it does not lower it.
- "Attention-seeking" delays the one thing that helps — a structured psychotherapy, with one consistent team.

Personality, trait, and disorder

Everyone has a personality. Most have some strong traits. A few have a disorder. The line is drawn by how pervasive, inflexible and harmful the pattern is, not by how odd it is.

Personality

- The enduring pattern of how a person thinks, feels, relates and behaves
- Shaped by temperament (inborn) and character (learned)
- Broadly stable across situations and through adult life
- Normal personality is FLEXIBLE: it adapts to the situation

Personality trait

- A strong, consistent feature: cautious, meticulous, reserved
- Can be useful in the right role: a surgeon benefits from being meticulous
- Causes no significant distress or impairment
- Having strong traits is NOT a disorder

Personality disorder

- Traits become INFLEXIBLE and PERVASIVE across most situations
- The pattern itself causes distress or impairs function
- Stable and long-standing, going back to adolescence
- Deviates markedly from the person's OWN cultural expectations

The whole diagnosis turns on three words: pervasive, inflexible, impairing. A trait that appears only in one setting, or that the person can switch off, is not a disorder.

The DSM-5-TR general criteria

A–F

Every personality disorder must FIRST meet these six general criteria. Only then do you ask which of the ten types fits. The pattern is enduring, pervasive, inflexible and stable, and judged against the patient's own culture.

A An enduring pattern

Inner experience and behaviour deviating markedly from the person's CULTURE, in ≥ 2 of: cognition, affectivity, interpersonal functioning, impulse control

B Inflexible & pervasive

The pattern is rigid and shows across many personal and social situations, not just one relationship or setting

C Distress or impairment

It leads to clinically significant distress, or impairment in social, occupational or other important areas of functioning

D Stable & long-standing

Onset goes back at least to adolescence or early adulthood. It is not a recent change

E Not another disorder

Not better explained by another mental disorder (e.g. not simply a mood or psychotic illness)

F Not substance / medical

Not due to a substance or another medical condition such as head injury

Students skip criteria D, E and F, yet they are exactly what separates a personality disorder from a mood episode, intoxication or a frontal-lobe lesion. Source: DSM-5-TR, 2022.

When it is NOT a personality disorder

Each of these can look like a personality disorder at one point in time. The history, not a single interview, tells them apart.

A state, not a trait

A depressive episode makes anyone dependent and pessimistic; mania makes anyone grandiose and reckless. Wait for a normal, euthymic baseline before judging personality.

Substance use

Intoxication and withdrawal can mimic almost every cluster: antisocial, borderline, paranoid. Diagnose the personality only when the person is abstinent.

An organic cause

A frontal-lobe tumour, dementia or head injury can cause a genuine, acquired personality CHANGE in adult life. New onset after 40 is organic until proven otherwise.

Developmental stage

Personality is still forming before 18. You can note traits, but the diagnosis is usually kept for adulthood, as adolescence is not yet a fixed pattern.

A cultural judgement

Criterion A is explicit: the pattern must deviate from the person's OWN culture. Reticence, deference or intense religious practice may be the norm, not a disorder.

A normal reaction

Grief, trauma and an abusive relationship cause understandable changes tied to the situation. They are reactions to circumstances, not a pervasive lifelong pattern.

Your most useful tool is a long-term history with collateral: is this who the person has ALWAYS been, across settings, or a change tied to a time, a substance or a relationship?

The three clusters

Ten disorders, three clusters by descriptive resemblance — a scaffold, not watertight compartments.

Cluster A

Odd / eccentric · "Weird"

Paranoid
pervasive distrust

Schizoid
detached, no desire for contact

Schizotypal
eccentric, odd beliefs

Cluster B

Dramatic / erratic · "Wild"

Antisocial
disregards others' rights

Borderline
unstable mood, identity, relationships

Histrionic
emotional, attention-seeking

Narcissistic
grandiose, needs admiration

Cluster C

Anxious / fearful · "Worried"

Avoidant
socially inhibited, fears rejection

Dependent
excessive need to be cared for

Obsessive–compulsive
orderly, controlling, rigid

How common are they?

Measure	Figure	Comment
Any personality disorder — community	≈ 10%	Roughly one adult in ten; up to half of psychiatric outpatients and most frequent attenders
Most prevalent type	OCPD	Obsessive–compulsive PD, about 2–8% of the population; often never reaches services
Borderline PD	≈ 1.5–2%	Up to 10–20% of psychiatric inpatients; mostly women in CLINICAL samples
Antisocial PD	≈ 1–3%	Men outnumber women roughly 3:1; concentrated in forensic and addiction settings
Age and course	Onset in youth	By definition begins by adolescence; Cluster B impulsivity tends to ease after the 30s
Mortality — borderline	Suicide ≈ 8–10%	Far above the general population; self-harm occurs in the majority at some point

Figures are community estimates and vary widely by method; clinical populations have far higher rates. The headline worth keeping: about one adult in ten, and borderline is lethal.

Trait, state, or disorder?

A 46-year-old accountant is meticulous, punctual and devoted to his work. Colleagues find him rigid, but he is respected, happily married and has kept close friends for decades.

Three weeks ago his father died. Since then he has been tearful, indecisive and clinging to his wife for reassurance, quite unlike himself. His GP wonders about a dependent or obsessive–compulsive personality disorder.

What is the correct assessment?

A Obsessive–compulsive personality disorder

B Dependent personality disorder

C Strong traits plus a normal grief reaction, so no disorder

D Two personality disorders, comorbid

Commit to an answer before the next slide.

Trait, state, or disorder?

ANSWER

A 46-year-old accountant is meticulous, punctual and devoted to his work. Colleagues find him rigid, but he is respected, happily married and has kept close friends for decades.

Three weeks ago his father died. Since then he has been tearful, indecisive and clinging to his wife for reassurance, quite unlike himself. His GP wonders about a dependent or obsessive–compulsive personality disorder.

What is the correct assessment?

A Obsessive–compulsive personality disorder

B Dependent personality disorder

C **Strong traits plus a normal grief reaction, so no disorder**

D Two personality disorders, comorbid

C — meticulousness is a useful trait, and the clinging is new grief tied to his loss.

His meticulousness is a lifelong TRAIT that causes no impairment: he is respected, married and has lasting friendships, so it fails Criterion C. The dependent picture is only three weeks old and clearly set off by his father's death, so it fails Criterion D (stable, long-standing) and is better explained as grief (Criterion E). Diagnosing a personality disorder here would label both a strength and a normal loss as illness. What would change the answer: a lifelong, pervasive pattern that started long before the death and impaired his work and relationships.

BMC
Batterjee Medical College

PART 2

Cluster A — odd and eccentric

Paranoid · schizoid · schizotypal, and where eccentricity, culture and the schizophrenia spectrum meet.

Paranoid personality disorder

Pervasive distrust — others' motives read as malevolent.

Threshold — ≥ 4 of 7



S	Suspects exploitation without basis
U	Unforgiving — persistently bears grudges
S	Spouse's or partner's fidelity doubted
P	Preoccupied with friends' loyalty
E	Enemy readings — threats seen in benign remarks
C	Confiding feared — reluctant to disclose
T	Threats to character perceived; reacts angrily

NOT psychosis — there are no delusions and no hallucinations.

Differential: paranoid schizophrenia (delusions + hallucinations), delusional disorder, or a realistic response to real persecution. Source: DSM-5-TR, 2022.

Schizoid personality disorder

DISTANT

A pervasive detachment from relationships with a narrow range of emotion. Needs ≥ 4 of seven. The person does not WANT relationships; this is not the fearful avoidance of Cluster C.

D Detached / no desire

Neither desires nor enjoys close relationships, including being part of a family

S Solitary by choice

Almost always chooses solitary activities

A Absent close friends

Lacks close friends or confidants other than first-degree relatives

T Temperature flat

Shows emotional coldness, detachment or flattened affectivity

I Indifferent to praise

Appears indifferent to the praise or criticism of others

T Taking little pleasure

Takes pleasure in few, if any, activities (anhedonia-like, but lifelong)

N No interest in sex

Little, if any, interest in sexual experiences with another person

Key contrast: the schizoid patient does not want company; the AVOIDANT patient (Cluster C) longs for it but is too afraid of rejection. Desire is the discriminator. Source: DSM-5-TR, 2022.

Schizoid — remember it by

Gollum

The Lord of the Rings



Withdrawn into his own world, cut off from people: the schizoid pattern.

- Lives isolated, with no real relationships
- Emotionally detached and solitary by preference
- Absorbed in his own inner world
- A memory hook, not a textbook case: Gollum also shows paranoid and odd traits

Character mnemonics are memory hooks, not diagnostic portraits. No real individuals are used.

Schizotypal personality disorder

ME PECULIAR

A pervasive pattern of social problems with sharp discomfort in relationships, plus odd thinking or perceptions and eccentric behaviour. Needs ≥ 5. Genetically, it sits on the schizophrenia spectrum.

M **Magical thinking**

Odd beliefs or magical thinking that affect behaviour, out of step with cultural norms

P **Paranoid ideation**

Suspiciousness or paranoid ideation

U **Unusual thinking / speech**

Odd thinking and speech: vague, over-elaborate, metaphorical or stereotyped

I **Ideas of reference**

Ideas of reference (not frank delusions of reference)

R **Rising social anxiety**

Excessive social anxiety that does not ease with familiarity and is tied to paranoid fears

E **Experiences, unusual**

Unusual perceptual experiences, including bodily illusions

C **Constricted affect**

Inappropriate or constricted affect

L **Lacks friends**

Lack of close friends or confidants other than first-degree relatives

A **Appearance eccentric**

Behaviour or appearance that is odd, eccentric or peculiar

This is the one that most resembles psychosis, but the experiences fall SHORT of frank delusions and hallucinations. Low-dose antipsychotics can help the cognitive–perceptual symptoms. Source: DSM-5-TR, 2022.

Cluster A — reading belief in context

Criterion A requires the pattern to deviate from the person's OWN culture. In our setting this judgement often decides whether Cluster A is diagnosed at all.

Not a disorder here

- Belief in the evil eye, jinn or the benefit of ruqyah where the community shares it
- Consulting a religious healer before, or alongside, a doctor
- Being reserved with strangers where that is culturally expected
- Intense, orthodox religious practice within the person's own tradition
- A realistic wariness in someone who HAS been persecuted or betrayed

Points toward Cluster A

- Beliefs the person's OWN family and community find strange and unusual
- Odd speech, eccentric appearance and ideas of reference beyond the cultural norm
- Lifelong detachment with no desire for the close family ties our culture expects
- Distrust so pervasive it isolates the person from their own community
- A pattern present since adolescence, across every setting, causing real harm

The practical test, as in the psychosis lecture: ask the family, "is this normal for him, or for people you know?" The community sets the baseline, not the clinician.

Which Cluster A disorder?

A 30-year-old man lives alone, chooses to work nights in a warehouse, and has no friends. He says he simply does not enjoy company and never has. He feels no loneliness and is not bothered by what others think of him.

He is not suspicious, has no odd beliefs or unusual perceptions, and his speech is ordinary. His affect is flat. His sister confirms he has been this way since his teens.

What is the most likely diagnosis?

A Schizotypal personality disorder

B Schizoid personality disorder

C Avoidant personality disorder

D Paranoid personality disorder

Commit to an answer before the next slide.

Which Cluster A disorder?

ANSWER

A 30-year-old man lives alone, chooses to work nights in a warehouse, and has no friends. He says he simply does not enjoy company and never has. He feels no loneliness and is not bothered by what others think of him.

He is not suspicious, has no odd beliefs or unusual perceptions, and his speech is ordinary. His affect is flat. His sister confirms he has been this way since his teens.

What is the most likely diagnosis?

A Schizotypal personality disorder

B **Schizoid personality disorder**

C Avoidant personality disorder

D Paranoid personality disorder

B — lifelong detachment with no DESIRE for relationships and no odd beliefs.

Schizoid is correct: detachment, a preference for being alone, flat affect and no wish for closeness, present since adolescence. Schizotypal (A) fails because there are no odd beliefs, unusual perceptions or eccentric speech. Avoidant (C) fails on the key point: the avoidant patient DESIRES relationships but fears rejection, whereas this man feels no loneliness and does not want them. Paranoid (D) fails because there is no suspiciousness. The word that decides it is DESIRE: its absence makes this schizoid, not avoidant.

BMC
Batterjee Medical College

PART 3

Cluster B — dramatic and erratic

Antisocial · borderline · histrionic · narcissistic: the cluster you will meet most, and the one that most tests you.

Antisocial personality disorder

CORRUPT

A pervasive disregard for and violation of others' rights since age 15. Diagnosis needs age ≥ 18, ≥ 3 of the features, AND evidence of CONDUCT DISORDER before age 15. “Psychopathy” adds callous, predatory traits.

C Conformity to law lacking

Repeatedly performs acts that are grounds for arrest

O Obligations ignored

Consistent irresponsibility: work and money obligations not met

R Remorselessness

Lacks remorse; shrugs off or makes excuses for hurting others

R Recklessness

Reckless disregard for the safety of self or others

U Underhandedness

Deceitful: repeated lying, use of aliases, conning others for profit

P Planning deficit

Impulsivity or failure to plan ahead

T Temper

Irritable and aggressive: repeated fights or assaults

The two gatekeepers students forget: the patient must be at least 18, and there must be documented conduct disorder before 15. Before 18 the diagnosis is conduct disorder. Source: DSM-5-TR, 2022.

Antisocial — remember it by



Hannibal Lecter

The Silence of the Lambs

Charming and brilliant, yet without any remorse: the antisocial core.

- Disregards and violates the rights of others
- Deceitful and manipulative to get what he wants
- No remorse; makes excuses for the harm he causes
- Cold and predatory: the “psychopathy” end of the spectrum

Borderline personality disorder

AM SUICIDE

A pervasive pattern of unstable relationships, self-image and affect, with marked impulsivity, from early adulthood. Needs ≥ 5 of nine. The highest-yield diagnosis in this lecture, and the most lethal.

A Abandonment fear

Frantic efforts to avoid real or imagined abandonment

S Suicidal / self-harm

Recurrent suicidal behaviour, gestures or threats, or self-mutilation

I Impulsivity

Impulsivity in ≥ 2 self-damaging areas: spending, sex, substances, driving, bingeing

I Identity disturbance

Markedly and persistently unstable self-image or sense of self

E Emptiness

Chronic feelings of emptiness

M Mood instability

Affective instability: intense moods that shift over hours, not weeks

U Unstable relationships

Intense, unstable relationships alternating between idealisation and devaluation

C Control of anger poor

Inappropriate, intense anger or difficulty controlling it

D Dissociation / paranoia

Transient, stress-related paranoid ideation or severe dissociative symptoms

Noura, from the opening case, meets at least seven of these. Here, self-harm most often relieves unbearable emotion rather than aiming at death, but the suicide risk is real and high. Source: DSM-5-TR, 2022.

Borderline — remember it by



Tiffany

Silver Linings Playbook

Intense, stormy relationships and raw mood swings: borderline on screen.

- Idealises people, then devalues them: relationships flip fast
- Impulsive and emotionally volatile, hour to hour
- Terrified of being abandoned
- Acts on the feeling before the thought

Borderline — what the criteria do not show

The nine criteria list the signs. These two columns show what actually happens in the room, and why the diagnosis is so often missed or mishandled.

Splitting and the team

- SPLITTING: people are seen as all-good or all-bad, and the view flips fast
- The same split appears in the TEAM: one nurse is idealised, another blamed
- Staff end up arguing with each other about the patient, which is itself a clue
- Self-harm and threats often rise around separations and discharge
- The feeling the patient stirs in YOU is data: notice it, do not act on it

Why it is so often mishandled

- Written off as “attention-seeking”, which delays real treatment
- Over-medicated in crisis, piling up drugs that were never going to help
- Admitted repeatedly, when admission can REINFORCE crisis behaviour
- Confused with bipolar disorder (see the next slide)
- Comorbidity is the rule: depression, PTSD, eating and substance use disorders

The therapeutic stance: consistent, kind, clearly bounded. Name the pattern to the patient without blame, keep limits predictable, and plan for the crises rather than being surprised by them.

Borderline PD versus bipolar disorder

Two discriminators decide it: the TIME-COURSE and the TRIGGER.

	Borderline PD	Bipolar disorder
Mood timeframe	Hours — highly reactive	Days to weeks
Trigger	Interpersonal — rejection, conflict	Autonomous / biological
Baseline self	Chronic emptiness, unstable identity	Stable self between episodes
Sleep & energy	Disturbed by distress	Reduced NEED for sleep; raised energy
Self-harm	A core, chronic feature	Mainly during mood episodes
First-line treatment	PSYCHOTHERAPY (DBT, MBT)	Mood stabilisers / antipsychotics

> They can co-occur. Mood over HOURS and set off by a relationship points to borderline; sustained episodes over DAYS with a reduced need for sleep point to bipolar.

Histrionic personality disorder

PRAISE ME

A pervasive pattern of excessive emotion and attention-seeking from early adulthood. Needs ≥ 5 of eight. The emotion is genuine but shallow and changes fast, and the discomfort is worst when not the centre of attention.

P **Provocative behaviour**

Inappropriately seductive or provocative in appearance or behaviour

A **Attention, centre of**

Uncomfortable when not the centre of attention

S **Speech impressionistic**

Speech excessively impressionistic and lacking in detail

M **Make-up / appearance**

Consistently uses physical appearance to draw attention to self

R **Relationships overestimated**

Considers relationships more intimate than they actually are

I **Influenced easily**

Suggestible: easily influenced by others or by circumstances

E **Emotions shifting**

Rapidly shifting and shallow expression of emotions

E **Exaggerated emotion**

Self-dramatisation, theatricality and exaggerated expression of emotion

Overlaps with borderline (the dramatic affect) and narcissistic (the need to be noticed), but lacks the self-harm and identity collapse of borderline and the grandiosity of narcissism. Source: DSM-5-TR, 2022.

Narcissistic personality disorder

GRANDIOSE

A pervasive pattern of grandiosity, need for admiration and lack of empathy from early adulthood. Needs ≥ 5 of nine. The grandiosity often hides a fragile self-esteem that collapses under criticism.

G **Grandiose self-importance**

Exaggerates own achievements

A **Arrogant**

Haughty, arrogant attitudes

D **Dreams of success**

Fantasies of success and power

O **Others envied**

Envious, or feels others envy them

E **Empathy lacking**

Lacks empathy for others

R **Requires admiration**

Requires excessive admiration

N **Need to be special**

Believes self unique and special

I **Exploitative**

Exploits others for own ends

S **Sense of entitlement**

Strong sense of entitlement

The clinical trap is the "vulnerable" subtype: the same entitlement and fragility, but showing as hypersensitivity and shame rather than open arrogance, and often mistaken for depression. Source: DSM-5-TR, 2022.

Narcissistic — remember it by



Miranda Priestly

The Devil Wears Prada

Grandiose, demanding and cold to everyone beneath her: textbook narcissism.

- Grandiose sense of self-importance
- Demands admiration and special treatment
- Exploits those around her without a thought
- Strikingly lacking in empathy

Mood swings — which disorder?

A 23-year-old woman reports “mood swings” several times a day. They are always set off by something with a friend or her family, such as a delayed reply or a perceived slight, and settle within hours. Between them she feels empty and unsure who she is.

She has cut herself since she was 16, usually to “feel something” or stop a feeling. Her sleep is poor only when she is upset, and her energy has never been elevated. A previous doctor started valproate for “rapid-cycling bipolar.”

What is the most likely diagnosis?

A Bipolar II disorder

B Rapid-cycling bipolar disorder

C Borderline personality disorder

D Cyclothymia

Commit to an answer before the next slide.

Mood swings — which disorder?

A 23-year-old woman reports “mood swings” several times a day. They are always set off by something with a friend or her family, such as a delayed reply or a perceived slight, and settle within hours. Between them she feels empty and unsure who she is.

She has cut herself since she was 16, usually to “feel something” or stop a feeling. Her sleep is poor only when she is upset, and her energy has never been elevated. A previous doctor started valproate for “rapid-cycling bipolar.”

What is the most likely diagnosis?

A Bipolar II disorder

B Rapid-cycling bipolar disorder

C **Borderline personality disorder**

D Cyclothymia

C — mood shifts that last hours, set off by relationships, with emptiness and self-harm since adolescence.

This is borderline personality disorder. The mood shifts last HOURS and are always triggered by a relationship; there is chronic emptiness, identity disturbance and self-harm that relieves emotion, all since 16. Bipolar of any kind (A, B) needs sustained episodes of DAYS to weeks and, for mania or hypomania, a reduced NEED for sleep with high energy; neither is present. Cyclothymia (D) also needs sustained hypomanic and depressive periods over two years, not hour-to-hour swings. The valproate treats a diagnosis she does not have; what she needs is a structured psychotherapy.

BMC
Batterjee Medical College

PAUSE

Break — 10 minutes

After the break: Cluster C, then why these disorders happen, how to assess them, and what actually helps.

BMC
Batterjee Medical College

PART 4

Cluster C — anxious and fearful

Avoidant · dependent · obsessive–compulsive: the cluster that suffers quietly and is most likely to ask for help.

Avoidant personality disorder

AFRAID

A pervasive pattern of social inhibition, feeling inadequate and being very sensitive to criticism, from early adulthood. Needs ≥ 4 of seven. The person LONGS for closeness but is too afraid of rejection to risk it.

A Avoids occupational contact

Avoids work that involves a lot of contact with people, for fear of criticism or rejection

R Rejection preoccupation

Preoccupied with being criticised or rejected in social situations

I Inhibited, feels inadequate

Holds back in new social situations because of feeling inadequate

F Fear of shame restrains intimacy

Holds back in close relationships for fear of being shamed or ridiculed

A Avoids new relationships

Unwilling to get involved unless certain of being liked

D Defective self-view & risk-averse

Sees self as socially inept or inferior, and avoids any personal risk for fear of embarrassment

Avoidant PD and generalised social anxiety disorder overlap so much that many see them as the same condition at different severities. The DESIRE for contact, blocked by fear, is what separates avoidant from schizoid. Source: DSM-5-TR, 2022.

Dependent personality disorder

RELIANCE

A pervasive, excessive need to be taken care of, leading to submissive, clinging behaviour and fears of separation, from early adulthood. Needs ≥ 5 of eight. Judge it carefully against cultural expectations of interdependence.

R **Reassurance to decide**

Needs advice for daily decisions

L **Others take charge**

Others assume major responsibilities

A **Helpless when alone**

Feels unable to cope alone

C **Replaces lost support**

Urgently seeks a new carer when one ends

E **Disagreement is hard**

Fears disagreement loses support

I **Struggles to initiate**

Lacks confidence to start alone

N **Support at any cost**

Goes to great lengths to get support

E **Fears being left**

Preoccupied with fears of being left

Cultural caution for our setting: interdependence within the family is the norm, not a disorder. Diagnose only when the dependence is EXCESSIVE for the person's own culture and clearly impairs them. Source: DSM-5-TR, 2022.

Obsessive–compulsive personality disorder

LAW FIRMS

A pervasive preoccupation with order, perfectionism and control, at the cost of flexibility, openness and efficiency, from early adulthood. Needs ≥ 4 of eight. NOT the same as OCD (see the next slide).

L **Loses the point (lists & rules)**

Preoccupied with details, rules, lists and order to the point the main activity is lost

W **Workaholism**

Devotion to work and productivity that crowds out leisure and friendships

I **Inability to delegate**

Reluctant to delegate unless others do things their exact way

M **Miserly**

Miserly with money, hoarding it for future disasters

A **Ability to complete lost**

Perfectionism that interferes with task completion

F **Fixed morally**

Over-conscientious and inflexible about matters of morality and ethics

R **Reluctant to discard**

Unable to throw away worn-out or worthless objects

S **Stubborn / rigid**

Rigidity and stubbornness

OCPD is ego-SYNTONIC: the patient sees nothing wrong and others complain. OCD is ego-DYSTONIC: the patient is tormented by their own symptoms. Different disorder, confusingly similar name. Source: DSM-5-TR, 2022.

Two distinctions students must get right

Each pairs a Cluster C disorder with the condition it is confused with. In each case the key difference is one idea.

OCPD versus OCD

- OCPD: a pervasive PERSONALITY style of order and control
- OCD: an anxiety disorder with intrusive OBSESSIONS and COMPULSIONS
- OCPD is EGO-SYNTONIC: the patient approves of their own standards
- OCD is EGO-DYSTONIC: the patient resists and is distressed by the symptoms
- A patient can have both, but most with OCPD have no true obsessions

Avoidant versus schizoid / social anxiety

- Avoidant WANTS relationships but fears rejection; schizoid does NOT want them
- Wanting closeness is the single thing that separates it from schizoid
- Avoidant PD and generalised social anxiety disorder largely overlap
- Think of them as the same difficulty across a spectrum of severity
- SSRIs and CBT help the social anxiety component in both

If one line survives: ego-syntonic (OCPD) versus ego-dystonic (OCD), and DESIRE for contact (avoidant) versus its absence (schizoid). Those two ideas answer most Cluster C exam questions.

OCPD or OCD?

A 38-year-old manager is referred because colleagues cannot work with him. He insists every report follow his exact format, refuses to delegate, stays late re-checking others' work, and has not taken leave in years. He keeps decades of receipts “in case.”

He sees no problem; everyone else is simply careless. He has no intrusive thoughts, performs no rituals to relieve anxiety, and is not distressed by his own behaviour, only by other people's standards.

What is the most likely diagnosis?

A Obsessive–compulsive disorder (OCD)

B Obsessive–compulsive personality disorder (OCPD)

C Generalised anxiety disorder

D Autism spectrum disorder

Commit to an answer before the next slide.

OCPD or OCD?

A 38-year-old manager is referred because colleagues cannot work with him. He insists every report follow his exact format, refuses to delegate, stays late re-checking others' work, and has not taken leave in years. He keeps decades of receipts “in case.”

He sees no problem; everyone else is simply careless. He has no intrusive thoughts, performs no rituals to relieve anxiety, and is not distressed by his own behaviour, only by other people's standards.

What is the most likely diagnosis?

A Obsessive–compulsive disorder (OCD)

B **Obsessive–compulsive personality disorder (OCPD)**

C Generalised anxiety disorder

D Autism spectrum disorder

B — a pervasive, EGO-SYNTONIC style of control, with no obsessions or compulsions.

This is OCPD: preoccupation with rules and order, perfectionism that stalls the task, over-devotion to work, inability to delegate, hoarding and rigidity, and, crucially, he sees nothing wrong (ego-syntonic). OCD (A) needs true obsessions and compulsions that the patient finds distressing and tries to resist (ego-dystonic); he has none. Generalised anxiety (C) is pervasive worry, not a controlling personality style. Autism (D) would show lifelong social-communication difference and restricted interests from early childhood, not described here. The whole answer rests on ego-syntonic versus ego-dystonic.

BMC
Batterjee Medical College

PART 5

Why they happen, and how to assess

Aetiology — temperament, genes and adversity · the assessment · culture, family and gender in our setting · comorbidity and risk.

Aetiology — nature and nurture together

No personality disorder has a single cause. An inborn temperament meets an early environment, and the disorder grows out of how the two interact.

The vulnerability (nature)

- Heritability of personality traits is substantial — roughly 40–60%
- Cluster A shares genetic loading with the SCHIZOPHRENIA spectrum
- Cluster B (especially borderline, antisocial) runs with impulsivity and mood instability in families
- Inborn TEMPERAMENT — reactivity, emotionality, novelty-seeking
- Neurobiology: amygdala–prefrontal dysregulation in borderline; low arousal in antisocial

The environment (nurture)

- Childhood ABUSE and neglect — strongly linked to borderline PD
- Insecure or disrupted ATTACHMENT in early life
- Inconsistent, harsh or chaotic parenting; early loss
- Trauma, and growing up amid violence or instability
- Culture and social environment shape how traits are expressed and judged

Use this model as you would in psychosis: it answers the family's "what did we do wrong?" The vulnerability was already there, hardship shaped it, and no single cause or blame explains it all.

Assessing for a personality disorder

You make the diagnosis over time, from several sources — never from one distressing encounter.

Think longitudinally

Ask about the pattern across the person's whole adult life and in different settings. The key question: "Has this always been how you are?"

Get collateral

Family and old friends see the pattern the patient cannot. Here the family is usually present and willing — use them, with consent.

Not in crisis

Do NOT diagnose during an acute crisis, a mood episode, intoxication or withdrawal. Re-assess at a calm baseline first.

Use structured tools

Validated interviews and screens (SCID-5-PD, IPDE) help, but support your judgement — they do not replace it.

Judge against culture

Apply Criterion A: is this pattern unusual for the PATIENT'S own culture, or normal within it?

Assess risk early

Check suicide, self-harm, harm to others and exploitation risk at first contact and at every crisis, especially in Cluster B.

Diagnosing is something you do over time: you DIAGNOSE a personality disorder by watching a pattern, not by reacting to one difficult consultation.

Culture, family and gender in our setting

Personality is the most culture-bound diagnosis, because Criterion A is a cultural judgement. Three cautions matter most in our practice.

Where culture changes the reading

- Interdependence within the family is the NORM — do not read it as dependent PD
- Deference to elders and defined gender roles are cultural, not pathological
- Belief in jinn, the evil eye and ruqyah is shared — not schizotypal
- Modesty and reticence with strangers may be expected, not avoidant
- The baseline is the patient's own community, judged with the family's help

Where our setting adds risk

- Stigma is severe — a "personality disorder" label can affect marriage and standing
- Self-harm and suicide may be hidden from shame; ask directly and privately
- Record sensitively — these notes carry real social consequences here
- Family involvement is a strength, but confidentiality and consent still apply
- Women with borderline PD may present with somatic complaints rather than emotion

Keep identifying details on-device and anonymise before any external search. The social cost of disclosure is higher here than textbooks written elsewhere assume.

Comorbidity and risk

Personality disorders rarely come alone. The comorbidity is usually what brings the patient in, and the risk is what you must not miss.

Comorbidity is the rule

- MOOD and ANXIETY disorders — the commonest reason for presenting
- SUBSTANCE use disorders — especially with Cluster B
- EATING disorders and PTSD — particularly with borderline PD
- Other personality disorders — the clusters overlap heavily
- Treating only the comorbidity, while ignoring the personality disorder, often fails

Risk you must assess

- SUICIDE — borderline PD carries roughly an 8–10% lifetime rate
- SELF-HARM — present in the majority with borderline at some point
- HARM TO OTHERS — antisocial PD, and jealous/paranoid presentations
- EXPLOITATION and victimisation — dependent and histrionic patients
- Risk fluctuates with crises — reassess it every time, do not assume

Do not assume that repeated self-harm means low suicide risk. The opposite is true: a history of self-harm is one of the strongest predictors of later suicide.

BMC
Batterjee Medical College

PART 6

Management and prognosis

General principles · the evidence-based psychotherapies · the limited role of medication · managing crisis · prognosis · the non-specialist's survival guide.

General principles of management

What helps most is not a named therapy but a consistent, clearly-bounded relationship. These principles apply in any specialty.

The stance

- Be consistent, reliable and clearly BOUNDED — the same limits every time
- Validate the distress without endorsing harmful behaviour
- Name the diagnosis honestly and without blame; psychoeducate the patient
- Treat comorbid mood, anxiety and substance disorders actively
- Work to a long time-frame — change is measured in years, not weeks

The structure

- One coordinating clinician or team — avoid fragmented, split care
- An agreed, written crisis plan the patient keeps and helps to make
- A clear plan for self-harm that avoids reflexive, repeated admission
- Involve the family appropriately, with the patient's consent
- Look after the team: these patients evoke strong reactions — supervise and share

These principles alone — consistency, boundaries, honesty, continuity — are "good psychiatric management" (GPM), which works as well as specialist therapies for many borderline patients.

The evidence-based psychotherapies

Psychotherapy is first-line for personality disorders, borderline especially. Recognise these structured models — you are not expected to deliver them.

DBT

Dialectical behaviour therapy (Linehan). The strongest evidence base in borderline PD; teaches distress tolerance and emotion regulation.

MBT

Mentalisation-based therapy. Builds the capacity to read one's own and others' mental states — the ability that collapses under emotional arousal.

TFP

Transference-focused psychotherapy. Uses the patient–therapist relationship to integrate the split all-good / all-bad views of self and others.

Schema therapy

Reworks lifelong maladaptive "schemas" formed in childhood. Evidence across several personality disorders, not only borderline.

STEPPS

A group skills programme designed to run ALONGSIDE usual care — practical where specialist individual therapy is scarce.

GPM

Good psychiatric management. Structured generalist care that matches intensive therapies for many patients — the realistic option in most services.

Common thread: all are structured, consistent, time-limited and built on the relationship. Where specialist therapy is not available, GPM done well is a real, evidence-based treatment — not a second-best stopgap.

The limited role of medication

No medication treats a personality disorder itself. Drugs are add-ons for specific symptoms or comorbid disorders — and they are easy to misuse here.

Reasonable, targeted use

- Treat comorbid DEPRESSION, anxiety or psychosis on their own merits
- SSRIs — for comorbid depression/anxiety and some impulsive aggression
- Second-generation antipsychotics — short-term for severe agitation or transient psychotic-like symptoms
- Mood stabilisers — limited, mixed evidence for affective instability and impulsivity
- Low-dose antipsychotics — cognitive–perceptual symptoms in schizotypal PD

How medication is misused

- Prescribing AS IF a drug could treat borderline PD itself — it cannot
- POLYPHARMACY accumulated across crises and never reviewed
- BENZODIAZEPINES — avoid: disinhibition, dependence, overdose risk
- Large or lethal-in-overdose supplies (e.g. TCAs) to a self-harming patient
- Starting drugs in crisis and never stopping them — review and deprescribe

NICE is clear: no drug is licensed to treat borderline PD itself. Medication targets a named comorbid disorder or a specific symptom, at the lowest dose for the shortest time, and is reviewed.

Managing the borderline crisis

The emergency visit is where most harm is done — by over-reacting or by dismissing. A planned, measured response is the treatment.

In the crisis

- Assess THIS episode's risk on its own — never assume it is like the last
- Stay calm, validate the distress, and keep the boundary — both at once
- Treat injuries and address immediate safety; involve the crisis team
- Prefer brief, structured contact over reflexive admission
- Use the patient's own crisis plan if one exists — and make one if not

Between crises

- Engage in a structured psychotherapy — DBT or an available equivalent
- One consistent team and an agreed, written crisis plan
- Treat comorbidity; review and rationalise medication
- Psychoeducation for patient and, with consent, family
- Hold realistic hope — and say so: most improve substantially over time

Admission has a place — acute, high, changed suicide risk — but long or repeated admission can reinforce crisis behaviour and erode function. Keep it brief and purposeful, not long and open-ended.

Prognosis and course

Disorder	Course	Outlook
Borderline PD	Impulsivity and self-harm fade a lot through the 30s and 40s	GOOD — roughly 85% reach remission over 10 years; hopeful
Antisocial PD	Offending and aggression often lessen ("burn out") after the 40s	Guarded — limited treatment response; manage comorbidity
Cluster A	Usually stable and long-lasting; schizotypal may move toward the schizophrenia spectrum	Guarded; rarely seeks treatment
Cluster C	The most stable over time, but the most treatable with therapy	Fair to good — responds to CBT and psychodynamic work
General rule	Most personality disorders become LESS severe with age; relationships may stabilise	Therapeutic pessimism is usually unjustified

The headline for patients and families: these are not fixed life sentences. Borderline PD in particular has one of the better long-term trajectories in psychiatry, and the message of realistic hope is itself therapeutic.

You will not run a DBT programme, but you will meet these patients in every specialty. Six rules that help you and them.

Do not diagnose in anger

If a patient makes you reach for "personality disorder," pause. Diagnose from a pattern over time, not from your irritation.

Hold the boundary, keep the warmth

Consistency is kindness. Clear, predictable limits without hostility are what helps — not coldness, and not endless flexibility.

Take every crisis seriously

Chronic self-harm does not mean safe. Risk-assess each presentation; a self-harm history RAISES suicide risk.

Resist the prescription reflex

Do not medicate distress itself. Avoid benzodiazepines and polypharmacy; treat a named comorbidity and review.

Coordinate, do not split

One plan, one team. When staff start disagreeing about a patient, suspect splitting and talk to each other.

Refer, and hold hope

Direct the patient to structured psychotherapy, and say honestly that most people improve — realistic hope helps.

If a student keeps one slide from this lecture, make it this one: it converts the whole two hours into six things they can actually do on any ward, in any specialty.

First-line treatment?

Noura, from the opening case, is now seen in a calm outpatient clinic. Her diagnosis is clearly borderline personality disorder: a long-standing, pervasive pattern meeting seven of the nine criteria, with repeated self-harm.

She asks what will actually help. A previous doctor gave her three medications across her crises, none reviewed. She has never been offered a psychological therapy.

What is the most appropriate first-line treatment?

A Add a mood stabiliser for affective instability

B A benzodiazepine for crises

C A structured psychotherapy such as DBT

D Long-term inpatient admission

Commit to an answer before the next slide.

First-line treatment?

Noura, from the opening case, is now seen in a calm outpatient clinic. Her diagnosis is clearly borderline personality disorder: a long-standing, pervasive pattern meeting seven of the nine criteria, with repeated self-harm.

She asks what will actually help. A previous doctor gave her three medications across her crises, none reviewed. She has never been offered a psychological therapy.

What is the most appropriate first-line treatment?

A Add a mood stabiliser for affective instability

B A benzodiazepine for crises

C **A structured psychotherapy such as DBT**

D Long-term inpatient admission

C — structured psychotherapy is the evidence-based first-line treatment for borderline PD.

Psychotherapy — DBT, MBT or an available structured option such as GPM — is first-line, with the best evidence for cutting self-harm and improving function. A mood stabiliser (A) has only limited, mixed evidence and treats a diagnosis she does not have; no drug treats borderline PD itself. A benzodiazepine (B) should be avoided — it causes disinhibition, dependence and overdose risk in a self-harming patient. Long-term admission (D) can reinforce crisis behaviour and erode function; admission should be brief and purposeful. The plan also means reviewing and cutting back her three unmonitored medications.

Board-style questions

#	Question	Answer
1	Which general criterion requires the pattern to begin by adolescence or early adulthood?	Criterion D — stable and of long duration, onset by adolescence / early adulthood
2	Schizoid versus avoidant — what single feature separates them?	DESIRE for relationships: avoidant wants them but fears rejection; schizoid does not want them
3	Mood shifts lasting hours, triggered by rejection, with self-harm since the teens — diagnosis?	Borderline personality disorder — not bipolar (which lasts days and needs elevated energy)
4	OCPD or OCD in a patient untroubled by his own perfectionism?	OCPD — ego-syntonic; OCD is ego-dystonic with true obsessions and compulsions
5	Two extra requirements for antisocial PD beyond the trait criteria?	Age ≥ 18, AND evidence of conduct disorder before age 15
6	First-line treatment for borderline PD?	Structured psychotherapy (e.g. DBT); medication is adjunctive and symptom-targeted only

Summary — eight things to remember

On what it is

- A personality disorder is an ENDURING, PERVASIVE, INFLEXIBLE pattern, present since adolescence, that impairs — not a difficult character
- Satisfy the GENERAL criteria first; rule out state, substance, organic and cultural explanations
- Judge every pattern against the patient's OWN culture — Criterion A is a cultural judgement
- Three clusters: A odd, B dramatic, C anxious — weird, wild, worried

On what to do

- Borderline is the high-yield one: hours-long reactive instability, emptiness, self-harm — and it is NOT bipolar
- Chronic self-harm RAISES suicide risk — risk-assess every single crisis
- PSYCHOTHERAPY is first-line; medication is adjunctive, symptom-targeted, and easily misused
- Prognosis is often GOOD, borderline especially — therapeutic pessimism is usually unjustified

If only one line survives the year, make it the first: this is an illness with a pattern and a treatment, not a verdict on a person's character.

References

Core references

- DSM-5-TR — American Psychiatric Association, 2022
- Kaplan & Sadock's Synopsis of Psychiatry
- Oxford Handbook of Psychiatry
- NICE CG78 — Borderline personality disorder: recognition and management
- NICE CG77 — Antisocial personality disorder

Specific points

- Linehan — dialectical behaviour therapy (DBT) for borderline PD
- Bateman & Fonagy — mentalisation-based therapy (MBT)
- Zanarini et al. (CLPS / MSAD) — long-term remission in borderline PD
- Prevalence given as community RANGES; clinical populations are enriched
- Where sources disagree, both figures are shown rather than one midpoint

Prevalence figures are given as published community ranges; clinical populations are far more enriched, and where sources disagree both numbers are shown.

BMC
Batterjee Medical College

Thank You

Dr. Hussain M. Khrad, MBBS, ABNP, ABAM, MA · Department of Psychiatry

Questions welcome. Sources for every figure are on the references slide.